GOVCARE EHR

CKD AI CLINICAL DECISION SUPPORT REPORT

Clinical decision support only - clinician review required

PATIENT & REPORT DETAILS

Patient	Demo Patient H03 0002
Patient number	PAT-NET-H03-0002
DOB	1945-07-14
Sex	female
Assessment ID	77e32068-4a4a-4e4f-8adf-4484e69bd654
Assessment time	2026-10-11 14:08:42.293538
Hospital	GovCare Demo Hospital 03
Department	Not recorded
Clinician / operator	Local Super Admin
Model	GOVCARE-CKD-LOGISTIC-1.0

CKD RISK SUMMARY

Predicted CKD risk	Risk category	AI confidence
98.2%	**URGENT REVIEW**	**96.3%**

CKD CLASSIFICATION & CLINICAL CODING

Suggested CKD classification	Stage 2
G category	G2
Suggested ICD code	N18.2
Clinical description	Chronic kidney disease, stage 2
Terminology system	http://hl7.org/fhir/sid/icd-10-cm
Basis	Reviewed eGFR 72 mL/min/1.73m²; chronicity and diagnosis require clinician confirmation
Coding status	AI_ASSISTED_SUGGESTION
Confirmation	Clinician confirmation required

A single eGFR result does not independently establish chronic kidney disease or chronicity. N18.6 (ESRD) is not inferred from eGFR alone.

○ G1	● G2	○ G3a	○ G3b	○ G4	○ G5

RENAL & ASSOCIATED FINDINGS

Parameter	Result	Unit	Source
eGFR	72	mL/min/1.73m²	PDF 02_partial_ckd_text_report.pdf 95% confidence
Serum creatinine	1.403	mg/dL	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence

Urine albumin/creatinine ratio (ACR)	55	mg/g	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence
Urine protein	8	mg/dL	MANUAL
Systolic blood pressure	146	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence
Diastolic blood pressure	92	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence
HbA1c	6.4	%	PDF 02_partial_ckd_text_report.pdf 95% confidence
BMI	27.4	kg/m²	PDF 01_full_ckd_text_report.pdf 95% confidence
Serum potassium	6	mmol/L	MANUAL
Hemoglobin	13.8	g/dL	PDF 05_no_ckd_values_report.pdf 95% confidence

CKD-RELATED CLINICAL RISKS & PREDICTIVE INTELLIGENCE

Sri Lankan Longitudinal Renal & Hemodialysis Data

Saved snapshot: 7e732699-7caf-4c2d-8fe2-3497f38fb758 | Captured 2026-10-11T08:38:42.523008Z. Export reads stored outputs; no inference is performed.

Renal Clinical Trajectory

INSUFFICIENT DATA | INSUFFICIENT DATA

Method: Longitudinal Trend

Current: Unavailable | Previous: Unavailable

Creatinine: Unavailable INSUFFICIENT DATA
Urea: Unavailable INSUFFICIENT DATA
Potassium: Unavailable INSUFFICIENT DATA
Hemoglobin: Unavailable INSUFFICIENT DATA
Albumin: Unavailable INSUFFICIENT DATA
URR: Unavailable INSUFFICIENT DATA

Clinical rule | LK-RENAL-MODULES-1.0 | NOT CLINICALLY VALIDATED

Review: AWAITING REVIEW | version 0

Directions compare the two latest valid values per domain using a 1% neutral band; this is not a prognosis or a G1-to-G5 forecast.
Dialysis timing, clinical context and measurement uncertainty require clinician interpretation.

Kidney Failure / G5D

INSUFFICIENT DATA | Insufficient clinical evidence

Method: Clinical Rule

Current: Unavailable | Previous: Unavailable

Clinical evidence: Chronicity: NOT_ESTABLISHED; Categories describe recorded measurements. CKD diagnosis requires clinician confirmation, chronicity and clinical context; G1/G2 alone do not establish CKD.; current eGFR-based clinical assistance, chronicity and diagnosis require clinician confirmation. | Verified: Not recorded

Clinical rule | LK-RENAL-MODULES-1.0 | NOT CLINICALLY VALIDATED

Review: AWAITING REVIEW | version 0

High creatinine alone does not establish G5D. This card reports recorded clinical evidence, not a diagnosis generated by AI.
No 2-year or 5-year kidney-failure probability is calculated.

Hyperkalemia Monitoring

NOT APPLICABLE | INSUFFICIENT DATA

Method: Longitudinal Research Model

Current: Unavailable mmol/L | Previous: Unavailable

LK-RENAL-HYPERK-1.0 | 1.0.0 | INTERNALLY VALIDATED

Review: AWAITING REVIEW | version 0

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.

Anemia / Hb Deterioration

NOT APPLICABLE | INSUFFICIENT DATA

Method: Persistence Baseline

Current: Unavailable g/dL | Previous: Unavailable

LK-RENAL-HB-1.0 | 1.0.0 | INTERNALLY VALIDATED

Review: AWAITING REVIEW | version 0

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
Learned candidates failed the gate against persistence. Carries forward the last measured Hb; no anemia-classifier probability is installed.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.

Dialysis Adequacy / URR

NOT APPLICABLE | INSUFFICIENT DATA

Method: Longitudinal Research Model

Current: Unavailable % | Previous: Unavailable

URR = ((Pre-HD Urea - Post-HD Urea) / Pre-HD Urea) x 100

LK-RENAL-URR-1.0 | 1.0.0 | INTERNALLY VALIDATED

Review: AWAITING REVIEW | version 0

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.

Renal Clinical Deterioration

INSUFFICIENT DATA | INSUFFICIENT DATA

Method: Transparent Composite

Current: Unavailable | Previous: Unavailable

Creatinine: Unavailable INSUFFICIENT DATA
Urea: Unavailable INSUFFICIENT DATA
Potassium: Unavailable INSUFFICIENT DATA
Hemoglobin: Unavailable INSUFFICIENT DATA
Albumin: Unavailable INSUFFICIENT DATA
URR: Unavailable INSUFFICIENT DATA

Clinical rule | LK-RENAL-MODULES-1.0 | NOT CLINICALLY VALIDATED

Review: AWAITING REVIEW | version 0

Directional composite – not a validated prognosis score.
Equal weights; two latest valid observations per domain; 1% neutral stability band. Missing domains are excluded, never scored as zero.
A directional percentage is not a disease probability.

Clinical Decision Support / Research Use. Outputs require clinician interpretation. Research-model outputs are not independently validated diagnoses.

SIX-MODULE EVIDENCE & CLINICIAN REVIEW

Renal Clinical Trajectory

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Kidney Failure / G5D

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Hyperkalemia Monitoring

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Anemia / Hb Deterioration

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Dialysis Adequacy / URR

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Renal Clinical Deterioration

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

PREDICTIVE CLINICAL INTELLIGENCE - LONGITUDINAL REVIEW

Live authorized EHR history at report generation time. Projected values are statistical extrapolations and are never stored or presented as laboratory observations.

Recorded G / A category	Latest eGFR	Latest creatinine	Data completeness
- / -	Unavailable	Unavailable	Unavailable

HISTORICAL eGFR & PROJECTED TRAJECTORY

No usable dated eGFR history is available.

Horizon	Current	Projected	Projected range	Trend	Reliability	Status / interval method
3 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
6 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
12 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED

EXPLAINABILITY & DATA-QUALITY LIMITATIONS

Renal safety: INSUFFICIENT DATA. Renal observations have unresolved date, unit, provenance, plausibility or conflicting-result issues. Reconcile the source records before forecasting. A usable eGFR result from the previous 90 days is unavailable. A usable creatinine result from the previous 90 days is unavailable. At least two dated eGFR measurements spanning 90 days are required by the longitudinal sufficiency policy. An active or recent admission requires acute illness and fluid-status review before long-term forecasting (GovCare 14-day context policy).

- MISSING CLINICAL DATE: A usable measurement date is required; upload time is never substituted.
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Medication context: No recent active prescription record was found. Medication use and nephrotoxic exposure are unknown; reconcile with the patient.

CLINICIAN-CONTROLLED RECOMMENDATIONS

- No dated eGFR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.
- No dated Urine ACR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.

Method: TREND PROJECTION · Model: trend-linear-2.0 · Validation: STATISTICAL_PROJECTION_NOT_CLINICALLY_VALIDATED · Local validation: NOT_APPLICABLE

Clinical decision support only. Forecasts and risk estimates are not autonomous diagnoses.

CLINICAL DATA USED FOR ASSESSMENT

Parameter	Result	Unit	Source	Coding

Urine albumin/creatinine ratio (ACR)	55	mg/g	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence	-
Age	81	-	CALCULATED	-
BMI	27.4	kg/m²	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 39156-5
Blood urea nitrogen (BUN)	9	mg/dL	MANUAL	-
Diastolic blood pressure	92	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 8462-4
Edema	1	-	MANUAL	-
Family history of kidney disease	1	-	MANUAL	-
Fasting blood sugar	320	mg/dL	MANUAL	-
eGFR	72	mL/min/1.73m²	PDF 02_partial_ckd_text_report.pdf 95% confidence	LOINC 48642-3
HbA1c	6.4	%	PDF 02_partial_ckd_text_report.pdf 95% confidence	LOINC 4548-4
Hemoglobin	13.8	g/dL	PDF 05_no_ckd_values_report.pdf 95% confidence	LOINC 718-7
NSAID use	1	-	MANUAL	-
Previous acute kidney injury	1	-	MANUAL	-
Urine protein	8	mg/dL	MANUAL	-
Serum creatinine	1.403	mg/dL	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence	LOINC 2160-0
Serum potassium	6	mmol/L	MANUAL	-
Systolic blood pressure	146	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 8480-6

KEY FACTORS IN THIS MODEL OUTPUT

Contribution direction describes this model's prediction, not whether a clinical value is healthy, safe or protective.

Factor	Value	Unit	Direction	Contribution
Fasting blood sugar	320	mg/dL	Increases prediction	1.8731
Urine protein	8	mg/dL	Increases prediction	1.5332
Serum creatinine	1.403	mg/dL	Reduces prediction	-0.8201
Edema	1	-	Increases prediction	0.5688
Blood urea nitrogen (BUN)	9	mg/dL	Reduces prediction	-0.5380
Family history of kidney disease	1	-	Increases prediction	0.3341
Serum potassium	6	mmol/L	Increases prediction	0.2145
Systolic blood pressure	146	mmHg	Increases prediction	0.1761
Urine albumin/creatinine ratio (ACR)	55	mg/g	Increases prediction	0.1296

RECOMMENDED CLINICAL REVIEW

1. Prompt clinician review is recommended.
2. Correlate with current renal function, urine findings, and clinical examination.

CLINICIAN REVIEW

Suggested classification	Stage 2
Suggested code	N18.2
Coding decision	[ ] Confirm [ ] Modify [ ] Reject
Final clinician-selected code	_____
Clinical notes	_____ _____
Reviewed by / date	_____

This output does not diagnose disease or replace professional medical judgement. AI-assisted coding must be reviewed before a confirmed FHIR Condition is created.